

PATIENT ROLE CARDS

Wilderness First Aid — Scenario Role-Play

FOR INSTRUCTORS:

appropriate card and give it to the actor 5 minutes before the scenario begins. Ask the actor to read it privately and ask any questions before the

FOR ACTORS:

card carefully. You don't need medical knowledge — just follow the instructions. Stay in character. The more realistic you are, the better stu

SCENARIOS IN THIS PACKET:

1. Arterial Hemorrhage (Bleeding Control)

2. Shock Recognition

3. Open Femur Fracture with Shock

4. Spinal Injury

5. Traumatic Brain Injury

6. Heat Stroke

7. Severe Hypothermia

8. Anaphylaxis

9. Diabetic Emergency (Hypoglycemia)

10. Seizure (Post-Ictal State)

11. Snakebite

12. Multi-System Trauma (Capstone)

13. Frostbite Scenario

14. Massive Trauma / Impaled Object

15. Open Chest Wound

PATIENT ROLE CARD #1: Arterial Hemorrhage (Bleeding Control)

WHO YOU ARE:

Marcus, 28-year-old rock climber

WHAT HAPPENED (Your Backstory):

You were rappelling and your leg scraped a sharp rock outcrop, leaving a deep laceration on the inner thigh. There's a lot of blood — it's coming out in pulses with your heartbeat.

HOW TO ACT (Physical Behaviors):

1. Lying on the ground, leg extended, hand pressing on your thigh
2. Look pale and scared — breathe quickly
3. Moan/grunt with pain if anyone moves your leg or lifts your hand away
4. Tell students your hand is getting slippery from blood
5. If they apply direct pressure, visibly relax slightly but remain distressed
6. If no tourniquet within 5 min, start acting increasingly confused and weak

WHAT TO SAY (When Students Ask Questions):

Q: Name

A: Marcus Chen

Q: What happened

A: I was coming down the rock face and hit the edge really hard — there's so much blood

Q: Pain level

A: 9 out of 10 — it's throbbing, like it's pumping

Q: Allergies

A: None that I know of

Q: Medications

A: Nothing

Q: Medical history

A: Nothing — I'm healthy

Q: Last ate

A: Granola bar about 2 hours ago

VITAL SIGNS (For Reference):

HR 118 rapid/weak, RR 24, Skin pale/cool/moist, LOC Alert but anxious → deteriorating

■ DO NOT REVEAL (Unless Directly Asked):

The wound is approximately 8 cm long and deep; you can see pulsing blood

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

After 3 min if no tourniquet → start slurring words, say "I feel really dizzy." After 5 min → "I can't... I can't really..."

For actor use only — do not show to students

PATIENT ROLE CARD #2: Shock Recognition

WHO YOU ARE:

Priya, 42-year-old backpacker

WHAT HAPPENED (Your Backstory):

You've been hiking for 10 days and developed severe diarrhea and vomiting for the last 18 hours. You couldn't keep anything down. You feel terrible — weak, dizzy, your heart is racing.

HOW TO ACT (Physical Behaviors):

1. Sitting against a tree, but barely — lean heavily, look exhausted
2. Pale face, slightly moist skin (lightly mist face with water before scenario)
3. Can't stand without nearly fainting — say "I get really dizzy when I try to get up"
4. Breathing noticeably faster than normal (practice shallow fast breaths)
5. Hands should feel cool and slightly clammy (if actor willing)
6. Speech is slightly slowed and effortful

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I've been sick all night... vomiting and diarrhea... I can't keep anything in

Q: Symptoms

A: My heart is pounding really fast, I feel dizzy and weak, kind of cold even though it's warm out

Q: Last intake

A: A few sips of water 4 hours ago — couldn't keep it down

Q: History

A: No medical problems normally

Q: Medications

A: Just ibuprofen I've been taking for my hip

Q: Pain

A: My stomach hurts, cramping — about 6 out of 10

VITAL SIGNS (For Reference):

HR 126, RR 24, Skin pale/cool/moist, Cap refill 3-4 sec, LOC Alert but weak

■ DO NOT REVEAL (Unless Directly Asked):

You urinated only once in the last 12 hours and it was very dark

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

After good treatment: "That actually feels a little better." If untreated at 5 min: Become more confused, harder to keep eyes open.

For actor use only — do not show to students

PATIENT ROLE CARD #3: Open Femur Fracture with Shock

WHO YOU ARE:

Dale, 55-year-old mountain biker

WHAT HAPPENED (Your Backstory):

You hit a root at speed, went over the handlebars, and your left leg bent badly under you. You heard a crack and can't move it at all. It looks wrong — it's bent in the middle of the thigh where there is no joint.

HOW TO ACT (Physical Behaviors):

1. Lying on your back, absolutely DO NOT move your left leg
2. Left leg is slightly rotated outward and appears shorter
3. Extremely distressed and in severe pain — this is the worst pain of your life
4. Cry out sharply if anyone touches the leg without warning
5. Ask "am I going to be okay?" and "how long until the helicopter gets here?"
6. If asked to move leg: "NO — please, I can't"

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I went OTB [over the bars] hard — I heard a crack and I can't move my leg

Q: Pain

A: 12 out of 10 — I've never felt anything like this

Q: Can you feel your foot

A: Yes — it feels tingly and weird

Q: Can you wiggle toes

A: A little bit... it hurts to try

Q: History

A: I take a blood thinner — Warfarin — for a heart thing

Q: Last ate

A: Big breakfast 2 hours ago

VITAL SIGNS (For Reference):

HR 128 weak, RR 22, Skin pale/clammy, LOC Alert but in severe pain

■ DO NOT REVEAL (Unless Directly Asked):

You can see the bone through a tear in your shorts

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

After 2 minutes: "I'm feeling cold... and kind of dizzy." This is blood loss shock developing.

For actor use only — do not show to students

PATIENT ROLE CARD #4: Spinal Injury

WHO YOU ARE:

Alex, 22-year-old kayaker

WHAT HAPPENED (Your Backstory):

You were playing in a river wave and flipped. When you tried to roll up, you struck a submerged rock with the top of your head and felt a "zap" through your whole body. You're out of the water now but your neck hurts badly and your arms feel weird.

HOW TO ACT (Physical Behaviors):

1. Lying completely still on your back — you are AFRAID to move
2. DO NOT move your head at all — keep it perfectly still
3. Look up at ceiling/sky with wide, scared eyes
4. Neck is stiff — you will not turn your head
5. Hold your arms slightly away from body and say they feel "electric" or "tingly"
6. If anyone grabs your head: "Please be careful — I'm scared to move"

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I hit my head on a rock while kayaking. My neck hurts really bad and my arms feel strange — like electric.

Q: Can you feel your hands

A: Yes, but they tingle constantly — it doesn't stop

Q: Can you move your fingers

A: Yes (move slightly, grimace)

Q: Any back pain

A: My neck mostly — and a little between my shoulder blades

Q: Headache

A: Yes, a bad one

Q: Did you lose consciousness

A: I don't think so... it was all so fast

Q: History

A: Nothing, I'm totally healthy

VITAL SIGNS (For Reference):

HR 62 (bradycardia), RR 18, Skin warm/pink (paradoxically — neurogenic shock), LOC Alert/anxious

■ DO NOT REVEAL (Unless Directly Asked):

Your legs also feel slightly tingly — you didn't mention it because you hoped it was nothing

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

If not immobilized: "Every time someone touches me I get these electrical shocks down my arms."

For actor use only — do not show to students

PATIENT ROLE CARD #5: Traumatic Brain Injury

WHO YOU ARE:

Jamie, 31-year-old trail runner

WHAT HAPPENED (Your Backstory):

You were running a technical trail and fell, striking your head on a rock. You were briefly knocked out — your running partner says about 30 seconds. You woke up, and now you have the worst headache of your life and you keep forgetting things.

HOW TO ACT (Physical Behaviors):

1. Sitting, holding your head in both hands
2. Squint your eyes — light sensitivity (ask others not to shine lights in your face)
3. Speak slowly and with slight confusion
4. Ask the same question more than once: "What happened again?" even after being told
5. Occasionally say "I feel sick" and act nauseous
6. As scenario progresses: Get MORE confused, not less

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I was running and I fell... I hit my head... I'm not sure what happened exactly

Q: Do you know where you are

A: I think I'm on the trail... we're in the mountains, right? (some confusion)

Q: Do you know what day

A: Um... it's the weekend? I'm not sure

Q: Your name

A: Jamie (correctly)

Q: Any numbness

A: My left hand feels a little funny

Q: Headache

A: It's horrible — the worst I've ever had and it's getting worse

Q: Nausea

A: Yes, and I threw up once already

VITAL SIGNS (For Reference):

HR 58 (Cushing's — bradycardia is a red flag), RR 14 irregular, LOC Confused

■ DO NOT REVEAL (Unless Directly Asked):

Your pupils are slightly different sizes (actor: pretend — instructor will tell students if they check)

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

Mandatory — at 3-minute mark, become MORE confused and drowsier. Say "I'm really tired... I just want to sleep." This mimics herniation.

For actor use only — do not show to students

PATIENT ROLE CARD #6: Heat Stroke

WHO YOU ARE:

Jordan, 26-year-old trail runner

WHAT HAPPENED (Your Backstory):

You've been running a mountain race in extreme heat. You started feeling off about 20 minutes ago and now you're completely disoriented. Your body temp is through the roof.

HOW TO ACT (Physical Behaviors):

1. Lying on ground, arms slightly out, completely disoriented
2. Speak incoherently when asked — you can't form proper sentences
3. Skin is flushed and hot — act as if very hot (fan yourself weakly)
4. Combative if anyone touches you unexpectedly — "don't touch me!"
5. Eyes are partially open but unfocused
6. If students start cooling: Gradually become slightly more coherent

WHAT TO SAY (When Students Ask Questions):

Q: Name

A: Jordan... yeah... Jordan

Q: What happened

A: Running... I was running... hot... I don't feel right

Q: Any medical problems

A: [confused look, no coherent answer]

Q: How long have you been here

A: I... I don't know

Q: Are you thirsty

A: ...yeah [but do NOT swallow well — aspiration risk means no oral fluids]

VITAL SIGNS (For Reference):

HR 148, RR 28 rapid, Skin hot/flushed/moist, Temp estimated 105°F+, LOC Confused/combative

■ DO NOT REVEAL (Unless Directly Asked):

Jordan's running partner (instructor) reports race has been 4 hours in 98°F heat; Jordan hasn't taken water at last two aid stations

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

With cold immersion: After 4 min, become slightly more oriented. Without cooling: "I can't... I'm..." then unresponsive.

For actor use only — do not show to students

PATIENT ROLE CARD #7: Severe Hypothermia

WHO YOU ARE:

Tom, 48-year-old hiker

WHAT HAPPENED (Your Backstory):

You got separated from your group in the rain last night. You've been out all night in wet, cold conditions. You are severely hypothermic.

HOW TO ACT (Physical Behaviors):

1. Lying on ground, curled slightly, minimally responsive
2. Only respond to pain — not to voice (just groan if pinched)
3. Speech is impossible — only groans and faint sounds
4. Muscles appear rigid — don't bend easily
5. Appear dead-like — barely breathing (take very slow, shallow breaths)
6. If warmed gently and insulated — after several minutes you can begin to moan more

WHAT TO SAY (When Students Ask Questions):

Q: Early

A: No response to voice, only pain

Q: After 3-4 min rewarming

A: Begin to moan softly

Q: After 5+ min treatment

A: Weakly say "...cold... so cold"

VITAL SIGNS (For Reference):

HR 32 irregular (very slow), RR 6 barely perceptible, Skin cold/pale/mottled, LOC Unresponsive

■ DO NOT REVEAL (Unless Directly Asked):

You have been out here approximately 10 hours; your clothes are completely soaked through

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

If treated correctly: Slowly improve. If handled roughly: "Collapse" completely (instructor signals cardiac arrest)

For actor use only — do not show to students

PATIENT ROLE CARD #8: Anaphylaxis

WHO YOU ARE:

Sophie, 24-year-old hiker

WHAT HAPPENED (Your Backstory):

You stepped on a yellow jacket nest 8 minutes ago and were stung multiple times (at least 10 stings). You have a known bee allergy and your EpiPen is in your pack. You're scared.

HOW TO ACT (Physical Behaviors):

1. Sitting upright — you CANNOT lie down, it makes breathing worse
2. Breathe audibly — every breath has a faint wheeze
3. Scratch at your arms during scenario — hives are intensely itchy
4. Hold your throat: "It feels tight, like something is squeezing my throat"
5. Voice is slightly hoarse/strained
6. Say your EpiPen is in the front pocket of your pack

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: Bee stings — at least 10 of them — I'm allergic, I need my EpiPen!

Q: Are you breathing okay

A: Not really — my throat feels really tight

Q: Any hives

A: Yes — they're all over my arms and chest, they're so itchy

Q: Do you have EpiPen

A: In my pack — the front pocket — please hurry

Q: Other medications

A: Just birth control pills

Q: Last reaction

A: Once before — I had to go to the hospital

VITAL SIGNS (For Reference):

HR 136, RR 28 with wheeze, Skin flushed/hives/warm, BP dropping, LOC Alert/terrified

■ DO NOT REVEAL (Unless Directly Asked):

You feel your blood pressure dropping — you're getting dizzy when sitting

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

Without EpiPen at 2 min: Wheeze louder, become unable to speak full sentences. At 4 min: Instructor calls "patient is deteriorating rapidly"

For actor use only — do not show to students

PATIENT ROLE CARD #9: Diabetic Emergency (Hypoglycemia)

WHO YOU ARE:

Carlos, 34-year-old hiker/diabetic

WHAT HAPPENED (Your Backstory):

You're a Type 1 diabetic and took your morning insulin but barely ate breakfast because you were rushing. You've been hiking hard for 3 hours. Your blood sugar has crashed.

HOW TO ACT (Physical Behaviors):

1. Sitting, but swaying slightly — trouble staying upright
2. Speech is slurred and slow — like you're very drunk (but you're not)
3. Sweating profusely — dab water on forehead and neck before scenario
4. Hands are shaking — make them visibly trembling
5. Can't track conversations — trouble following what people say
6. If given sugar/juice: After 60-90 seconds, dramatically improve

WHAT TO SAY (When Students Ask Questions):

Q: Name

A: Car... Carlos (correctly but slurred)

Q: Are you diabetic

A: Yeah... yeah I have diabetes... I take insulin

Q: Did you eat

A: Not really... I was late... I had like half a granola bar

Q: Do you have sugar

A: I... maybe... my pack... front pocket

Q: How do you feel

A: Everything is spinning... I'm so sweaty... I feel drunk but I didn't drink

Q: Pain scale

A: I'm not in pain, I just feel WRONG

VITAL SIGNS (For Reference):

HR 116, RR 18, Skin pale/diaphoretic (sweating), LOC Verbal (confused/slurred)

■ DO NOT REVEAL (Unless Directly Asked):

You took your full dose of insulin this morning but only ate 200 calories

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

With sugar: Start "coming back" at 60-90 sec (pretend 10-15 min). Without treatment in 3 min: Become unresponsive.

For actor use only — do not show to students

PATIENT ROLE CARD #10: Seizure (Post-Ictal State)

WHO YOU ARE:

Casey, 19-year-old college student

WHAT HAPPENED (Your Backstory):

You are hiking with friends. You have no history of seizures. Without warning you had a generalized tonic-clonic seizure lasting about 90 seconds. You are now in the postictal state — confused, exhausted.

HOW TO ACT (Physical Behaviors):

1. The seizure has already happened when students arrive
2. You are on the ground, on your side, not fully conscious
3. Respond very slowly and confusedly to your name
4. When asked: Long pause before answering, confused
5. You have NO memory of the seizure or last 15 minutes
6. You are exhausted — keep eyes barely open

WHAT TO SAY (When Students Ask Questions):

Q: Name

A: Casey (correct, but very slowly)

Q: What happened

A: I... I don't know. I was just walking and then... I'm here. I don't know.

Q: Have you had seizures before

A: No... never. This has never happened.

Q: Do you hurt

A: My head... and my tongue hurts. Did I bite it?

Q: Medications

A: Birth control... that's all

Q: Medical history

A: Nothing. I'm healthy.

Q: Can you stand

A: I don't think so right now

VITAL SIGNS (For Reference):

HR 120 (post-ictal elevation), RR 20, Skin normal, LOC Verbal (post-ictal confusion)

■ DO NOT REVEAL (Unless Directly Asked):

You wet yourself during the seizure (instructor may or may not use this detail)

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

Over 5-10 minutes: Gradually become more oriented as post-ictal state clears. Still needs evacuation.

For actor use only — do not show to students

PATIENT ROLE CARD #11: Snakebite

WHO YOU ARE:

Riley, 35-year-old naturalist

WHAT HAPPENED (Your Backstory):

You reached under a ledge to retrieve your water bottle and a rattlesnake struck your right hand. You saw the snake clearly — triangular head, rattle on tail. That was 15 minutes ago. Your hand is swelling rapidly.

HOW TO ACT (Physical Behaviors):

1. Sitting very still — you've heard you shouldn't move
2. Right hand held low and away from body (below heart level)
3. Two fang marks visible on back of right hand (instructor marks with red marker)
4. Hand and forearm are visibly more swollen than other hand
5. Increasing pain at site — grimace if anyone touches the hand
6. Removed your ring before swelling got bad — mention if asked

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: A rattlesnake bit my right hand 15 minutes ago — I saw the snake, it definitely had a rattle

Q: How does it feel

A: It burns and aches really badly — and look at how much it's swollen already

Q: Are you having breathing trouble

A: Not yet — should I be?

Q: History

A: No medical problems

Q: Medications

A: Aspirin every day — low dose

Q: Allergies

A: Sulfa drugs

Q: Did you cut it or suck

A: [Embarrassed] "I tried to suck it for like a minute..."

VITAL SIGNS (For Reference):

HR 96, RR 18, Skin normal (no systemic envenomation signs yet), LOC Alert/anxious

■ DO NOT REVEAL (Unless Directly Asked):

Swelling has already spread to your elbow

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

After 5 minutes: "My vision feels a little weird... is that normal?" (signal of systemic envenomation)

For actor use only — do not show to students

PATIENT ROLE CARD #12: Multi-System Trauma (Capstone)

WHO YOU ARE:

Sam, 38-year-old hiker

WHAT HAPPENED (Your Backstory):

You fell off a 10-foot rock face, tumbling and landing on your back and hitting your head. You're conscious but confused. You have multiple injuries.

HOW TO ACT (Physical Behaviors):

1. Lying on ground — you moved slightly to a more comfortable position
2. Right arm held close to body and slightly bent — it hurts to straighten
3. Head has a small laceration that bled quite a bit (instructor applies moulage)
4. You're confused — similar to TBI but milder
5. You wince when anyone presses on your right ribcage
6. You're cold — slightly wet hair/face, it's a cold day

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I was climbing down and just... fell. I don't really remember landing.

Q: Your name

A: Sam... Sam Torres

Q: Where does it hurt

A: My arm, my ribs on this side, my head... kind of everywhere

Q: Can you feel your legs

A: Yes... they feel okay

Q: Headache

A: Yes, and I feel confused — I can't think straight

Q: LOC

A: Oriented to person and place, but NOT time

Q: Last ate

A: Lunch, maybe 3 hours ago

VITAL SIGNS (For Reference):

HR 104, RR 22, Skin cool, LOC Confused — Alert but not oriented ×4

■ DO NOT REVEAL (Unless Directly Asked):

Right shoulder is dislocated (not just arm sore); there may be a rib fracture on right

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

During assessment: "I'm really cold... and my chest really hurts when I breathe in deeply."

For actor use only — do not show to students

PATIENT ROLE CARD #13: Frostbite Scenario

WHO YOU ARE:

Morgan, 45-year-old skier

WHAT HAPPENED (Your Backstory):

You were caught in a whiteout on the mountain for 2 hours in extremely cold conditions. Your feet are very cold, your right foot especially — you can't feel your toes on the right side.

HOW TO ACT (Physical Behaviors):

1. Walking stiffly — you don't want to put full weight on right foot
2. Right boot is still on — you haven't taken it off because you're scared
3. Left foot is uncomfortable but functional
4. You're shivering slightly but functional — mild hypothermia
5. Anxious about your foot — "I'm scared to take my boot off"

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I was caught in the whiteout for a couple hours — my feet got really cold. My right foot — I can't feel my toes.

Q: Are you warm now

A: A little warmer, but I was shaking really badly for a while

Q: Can you feel right foot

A: No — the toes are completely numb. It felt like pins and needles before, now nothing.

Q: Can you walk

A: Sort of — it hurts if I put full weight on my right foot

Q: Last resort

A: I can walk if I have to — it's 6 miles to the car

Q: History

A: I have poor circulation from smoking — both feet usually get cold easily

VITAL SIGNS (For Reference):

HR 78, RR 16, Skin cool/pale from cold, LOC Alert x4

■ DO NOT REVEAL (Unless Directly Asked):

Right toes appear white and waxy when boot is removed (instructor: mark with face paint if possible)

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

6-mile ski out option → Do NOT rewarm. Actor: "So are you going to put my feet in water or what?"

For actor use only — do not show to students

PATIENT ROLE CARD #14: Massive Trauma / Impaled Object

WHO YOU ARE:

Devon, 29-year-old mountain biker

WHAT HAPPENED (Your Backstory):

You crashed and a broken branch impaled you in the left lateral abdomen. The branch is approximately 12 inches of a stick protruding from your left side. You are in severe pain but surprisingly stable.

HOW TO ACT (Physical Behaviors):

1. Lying still on your left side — the stick is on your left
2. ABSOLUTELY do not move — every little movement makes pain worse
3. Speaking through clenched teeth — severe pain but maintaining
4. Right hand on ground, stabilizing yourself
5. DO NOT let anyone touch the stick — scream/yell if anyone reaches for it
6. You're scared: "Please don't pull it out — I read you're not supposed to"

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: I crashed into a log pile... I know there's something in me... don't touch it

Q: Can you breathe

A: Yes... it hurts to breathe deep but I can breathe

Q: Pain

A: 10 out of 10 — but I can still think clearly

Q: History

A: I have Crohn's disease — I take humira

Q: Allergies

A: Penicillin

Q: Last ate

A: Breakfast, 3 hours ago. Nothing since.

Q: Are you bleeding

A: I don't know... I think so... there's blood on my shirt

VITAL SIGNS (For Reference):

HR 110, RR 20 shallow, Skin pale, LOC Alert x4

■ DO NOT REVEAL (Unless Directly Asked):

There is also abdominal guarding — your abdomen is rigid around the stick

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

At 3 minutes: "I'm feeling... kind of lightheaded. How long until help comes?"

For actor use only — do not show to students

PATIENT ROLE CARD #15: Open Chest Wound

WHO YOU ARE:

Blair, 51-year-old hunter

WHAT HAPPENED (Your Backstory):

You were field-dressing a deer and the knife slipped, causing a penetrating chest wound on the right side. You can hear a sucking sound when you breathe. You're sitting against a tree, breathing hard.

HOW TO ACT (Physical Behaviors):

1. Sitting upright — lying down makes breathing much harder
2. Breathing is visibly labored — shoulder rise/fall, neck muscle use
3. Make a soft sucking sound with each breath
4. Right hand is over the wound site
5. Increasing respiratory distress — lean forward, tripod position
6. Speak in short sentences only — can't take a deep breath

WHAT TO SAY (When Students Ask Questions):

Q: What happened

A: [short of breath] "Knife slipped... hunting... punctured my chest"

Q: Can you breathe

A: Barely... you can hear it

Q: Pain

A: 8/10... chest and shoulder

Q: History

A: Mild asthma... I have an inhaler

Q: Allergies

A: Morphine... makes me vomit

Q: Last ate

A: Lunch at noon

Q: Medications

A: Albuterol inhaler, blood pressure pill

VITAL SIGNS (For Reference):

HR 122, RR 28 labored, Skin pale/moist, LOC Alert but distressed

■ DO NOT REVEAL (Unless Directly Asked):

The wound makes a distinct sucking sound with each breath — air entering the chest

SCENARIO PROGRESSION (If Instructor Signals Time Pass):

Without 3-sided occlusive dressing at 2 min: Breathing worsens ("I can't breathe..."). With treatment: Slight improvement.

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